PathwayAI: questions and answers for after the demo

These notes are for discussion, not AI narration. They preserve the qualifications moved out of the speech. Do not describe a proposal, software check or formative review as an established clinical result or partnership.

Have you proved earlier diagnosis or cost savings?

No. The working tools support care preparation, cost/time recording and resource navigation. A prospective pilot would evaluate usefulness, access and downstream effects. We have not established a causal reduction in diagnostic delay, disability or spending.

Does each earlier diagnosis save about $1,900?

No. Hook's confirmed localized and disseminated groups had societal means of $1,307 and $3,251 in 2016 dollars, a $1,944 observed difference. That is not a causal estimate of money saved per earlier diagnosis or a PathwayAI intervention effect. Groups, severity and care experience differ.

Is the Dutchess $2.15 million figure actual county spending?

No. It is 1,060 reported cases in 2024 multiplied by the Hook overall study mean of $2,032, expressed in 2016 dollars. Orange uses 964 reported cases. The scenario assumes transfer of study experience. It does not adjust for inflation, underreporting or local case mix, and does not capture the full population's prediagnosis and long-term burden.

Why might the separate medical estimate exceed the societal illustration?

The Yu and Hook studies have different populations, methods, observation periods and dollar years. They are not components of one common total. The main illustration uses Hook; the optional Yu benchmark is separated and marked pending original-table verification. Do not add them or compare the gap as a real county cost difference.

What do the CMS figures mean?

They are annual Original Medicare payments per beneficiary by county of residence. Actual and standardized values are separate; standardization adjusts geographic payment rates, not health status. Hospital outpatient is not every form of ambulatory care. These are not all-payer spending, patient out-of-pocket bills, hospital charges or Lyme-specific

payments. We checked the supplied 22-row extract against the dictionary and arithmetic; we did not independently re-download the full CSV.

Are county health departments already using the app?

They are potential planning partners. The project owner reports that Lyme patients and a county health commissioner reviewed the working app. That feedback informed design; it is not county adoption, endorsement or a funded delivery agreement. Outreach messages are invitations, not commitments.

What can a county do with it today?

Review public context and use a planning brief to discuss a reported barrier, candidate action, responsible partners, needed resources and follow-up. For example, assess a small care-navigation pilot using verified existing clinics and assistance contacts. Local staff time, service capacity and funding must be agreed. A planning template does not establish a representative county need or automatically deliver a service.

Who benefits first?

Patients and caregivers can prepare reviewed summaries, record costs and time, and access support questions now. County and community teams are prospective planning users. Future secure, consented longitudinal research could connect patient-reported barriers with planning; that is not a current population-wide burden estimate.

Are private stories pooled or sent to county officials?

No automatic pooling or county submission occurs. Shared patient-report collection remains off. Private-session records and downloaded summaries are separate from public county data. Future collection needs explicit consent, secure storage, access and retention rules, duplicate checks and privacy-reviewed aggregate release.

Does the app diagnose Lyme or decide treatment?

No. It supports prevention information, care preparation and navigation. The clinician evaluates symptoms, exposure, testing and treatment. An extracted positive result is patient-reported and must be checked against the original laboratory report. Sudden facial drooping may indicate stroke and requires emergency attention; do not assume it is Lyme.

Is the uploaded self-check or photo analysis live?

No. The separate uploaded self-check was reviewed, not deployed. Its care-routing logic, including sudden facial weakness, needs clinical review. Tick/rash photo analysis, medical-report image redaction, Apple Health and Blue Button are not connected. Removing diagnostic wording alone would not establish regulatory status or clinical safety.

Is AI required or clinically validated?

The basic English organizer uses local rules. Optional external AI can organize notes; multilingual organization/translation requires permission. Outputs can contain errors and must be reviewed. Software tests and mocked API checks are not clinical or translation-quality validation. The bilingual summary supports preparation and does not replace a qualified medical interpreter.

Are community health centers free? Can anyone get assistance?

Health centers serve uninsured patients and offer sliding-fee discounts based on income and family size. Care is not automatically free; outside laboratories or referrals may have separate fees. Confirm charges, availability and assistance rules with the center. Access to health center services and eligibility for an insurance program are different questions. Emergency care may still be billed; cost concerns should not delay emergency care.

Does a short period of missed work establish disability-benefit eligibility?

No. Adult disability-based SSI/SSDI generally requires a medically determinable condition preventing substantial gainful activity that has lasted or is expected to last at least 12 months or result in death, alongside other program rules. The app provides preparation checklists, not eligibility decisions or applications. Keep long-term functional history distinct from a 30-day burden record.

How will this be funded? What is the budget?

The proposed model is free patient access supported by grants and community partnerships. Funding and partner commitments are not secured. We have not established a pilot budget. The next proposal should price staff time, resource verification, hosting, AI, interpretation/accessibility, secure data handling and evaluation for an agreed scope. Do not invent a funding request or return on investment.

What has actually been tested?

The latest full suite passed 68 software tests. A fictional flow also checked reported test results, timeline preservation, $225 paid, separate workdays/caregiver hours and data preservation when changing reading modes. The project owner reports working-app

reviews by people living with Lyme disease and a county commissioner. No quantified formal usability study, clinical validation or patient-outcome evaluation is claimed.

Are all data current and verified?

No single live feed is claimed. Sources have different reporting years. NYSDOH 2024 cases/rates and supplied Hook article text were checked; the historical CDC file covers 2019–2022. Census disability year labels in the regional report remain unresolved. The Yu original tables and the October 3 survey summary require further verification. Provenance and caveats are in the app's Sources page.

Why did you not show survey results in the demo? How can they help?

The demo focuses on public data and practical patient tools. Patient surveys can help identify burdens that official data may miss, such as difficulty finding care, missed work, unpaid caregiving and financial stress. They can guide which questions and support tools we prioritize.

The survey summary discussed included 33 voluntary respondents who reported a Lyme diagnosis. This small, self-selected sample cannot establish how common these problems are across a county or calculate county-wide costs. Before presenting a numerical finding, we need to verify the original responses, the question wording, the date and the number who answered that question. We have not verified that these survey results are used in the app's calculations.

Short answer: “Public data shows reported disease patterns. Patient feedback helps us understand what those numbers leave out—and what support people need. The survey can guide design, but it is not a representative county estimate.”

What comes next?

Recruit a small willing pilot; refine simple and language-accessible flows; verify local resources; measure task completion, referrals, appointments reached and assistance received. Design secure consented follow-up before shared reporting or longitudinal research. Use observed results to decide whether broader deployment is justified.